



Pulmonology Consultation Note – Evaluation of Pulmonary Nodules

Service: Respiriology / Pulmonary Medicine
Location: NorthStar Respiratory Clinic
Date of Service: 2022-06-10
Consultant: Pulmonology (network affiliate)
Referral Reason: Incidental pulmonary nodules
Referring Context: Chronic lymphadenopathy workup

****Reason for Consultation:**** Incidental pulmonary nodules on CT chest performed during lymphadenopathy evaluation.

****History of Present Illness:**** Patient denies cough, sputum, hemoptysis, dyspnea, wheeze, pleuritic pain, or recurrent pneumonia. No fevers or constitutional symptoms. He reports extensive travel but no known TB contacts. He is a never-smoker per chart and denies vaping or marijuana smoking. No significant occupational inhalational exposures documented.

****Review of Systems:**** Negative for weight loss, night sweats, chronic sinus symptoms, and chronic diarrhea. No history suggestive of sarcoidosis (no uveitis, erythema nodosum) documented in referral.

****Imaging Review:**** CT chest shows multiple bilateral pulmonary nodules ≤ 4 mm without spiculation or associated lymphadenopathy. Mild emphysematous background change noted; patient's exposure history does not clearly explain this and may represent minor imaging overcall or remote exposure.

****Infectious Workup:**** Sputum culture and AFB reportedly negative. TB considered unlikely in absence of symptoms and supportive imaging findings.

****Assessment:**** Very small pulmonary nodules most consistent with benign incidental findings (e.g., healed granulomas). Overall malignancy risk low given size and pattern.

****Plan:**** (1) Discussed surveillance imaging options; follow-up CT is ****optional**** and may be deferred to limit radiation. (2) Return PRN if respiratory symptoms develop. (3) Continue systemic evaluation for lymphadenopathy with primary/internal medicine teams.

****Patient Counseling:**** Reviewed red flags (new cough, hemoptysis, unexplained weight loss) and advised prompt reassessment if present.



Addendum / Reporting Notes

This document uses standardized institutional templates to emulate real-world medical record formatting for prototype testing. All names, identifiers, and institutional details are fictional. Clinical content is synthesized from the provided longitudinal summary.

If results appear internally inconsistent across documents, this is intentional to mimic typical real-world fragmentation. When discrepancies arise, the recommended approach is to reconcile using original source dates and the most recent verified medication/problem lists.